



American Board
of Internal Medicine®

Contributions of non-US-born Physicians to the US IM Workforce

ABMS Conference 2026– 9/16/2026

Giacomo Meille, Bradley Gray, Jonathan Vandergrift, Alicia Fernandez,
Salahuddin Kazi, Rebecca Lipner

Nothing to Disclose

Background

- Foreign-born physicians make up a large fraction of the IM workforce
- Immigration policy affects most foreign-born international medical graduates (IMGs) and some foreign-born US medical graduates (USMGs)
- Immigration pathways:
 - H-1B work visa
 - J-1 exchange visa → Conrad Waiver 3-year requirement to practice in underserved areas → H-1B

Recent changes in policy have disrupted physician immigration

- H-1B \$100,000 fee unless already in US (Sep 2025)
 - Vacated by court system (Jun 2026), may be appealed
 - May be replaced by \$103,000 fee on all cap-subject H-1B petitions (announced Aug 2026, only non-profit academically affiliated employers would be exempt)
- Restricted (no new J visa) for 39 countries (19 in Jun 2025, 20 in Jan 2026). Primarily in Africa, Middle East, Caribbean, include Iran, Nigeria, Venezuela.
 - An update on April 30th by US Citizenship and Immigration Services added an exemption for medical physicians

These policy changes have affected current and future US physicians

- Non-citizen IMG residency match rate for 2026 fell to 5-year low
- Physicians from restricted immigration countries placed on administrative leave by hospitals

Trump's Immigration Policy Sidelines Foreign Doctors Amid Shortage

Physicians from 39 countries are being pushed out of U.S. hospitals as a policy blocks their ability to work.

▶ Listen · 8:34 min

📺 Share full article



Summary

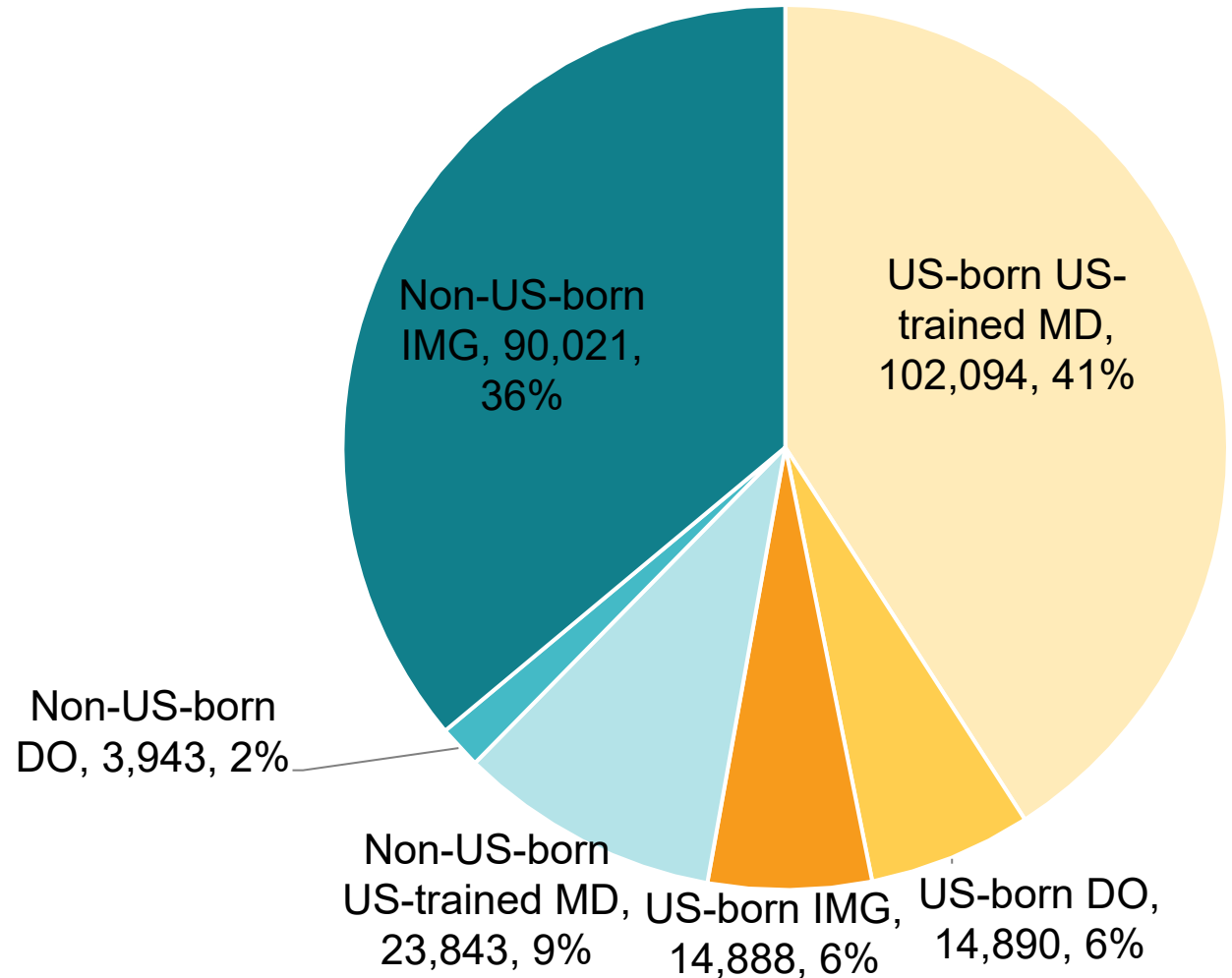
- **Research question:** How important are non-US-born physicians, especially IMGs who may be most affected by immigration policy, to the US IM workforce?
- **Analyses:**
 - 2026 workforce using ABIM data
 - 2024 patient analysis linking ABIM data to Medicare claims
- **Limitation:** Do not observe citizenship/visa status

Sample restrictions – Workforce analysis

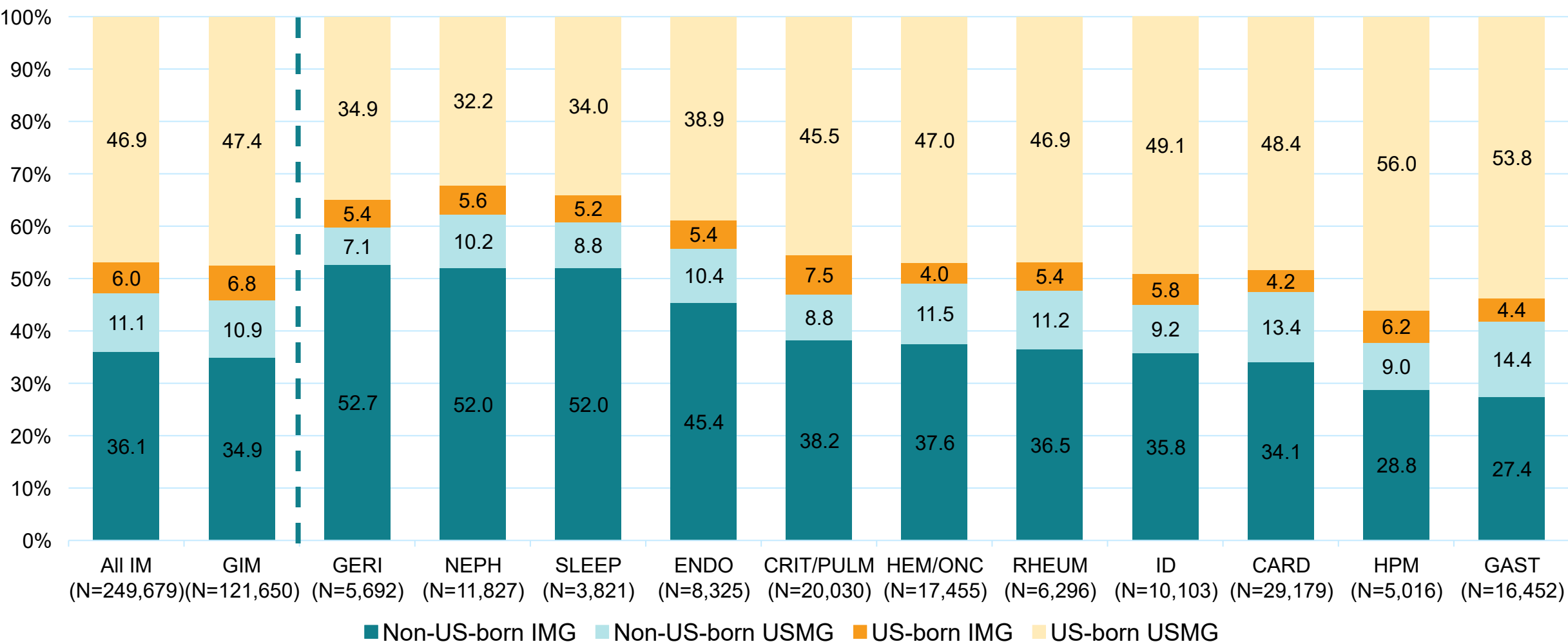
- Initial IM-certification in 1990 or later
- Active license
- Did not voluntarily deactivate all certifications
- Did not self-report as retired to ABIM
- Was not reported deceased to ABIM
- Has data on birth country or medical school country (99% of eligible sample)

The IM physician workforce was divided into four categories based on training and birth country

1. US-born USMGs
(US-trained MDs + DOs)
2. US-born IMGs
3. Non-US-born USMGs
(US-trained MDs + DOs)
4. Non-US-born IMGs

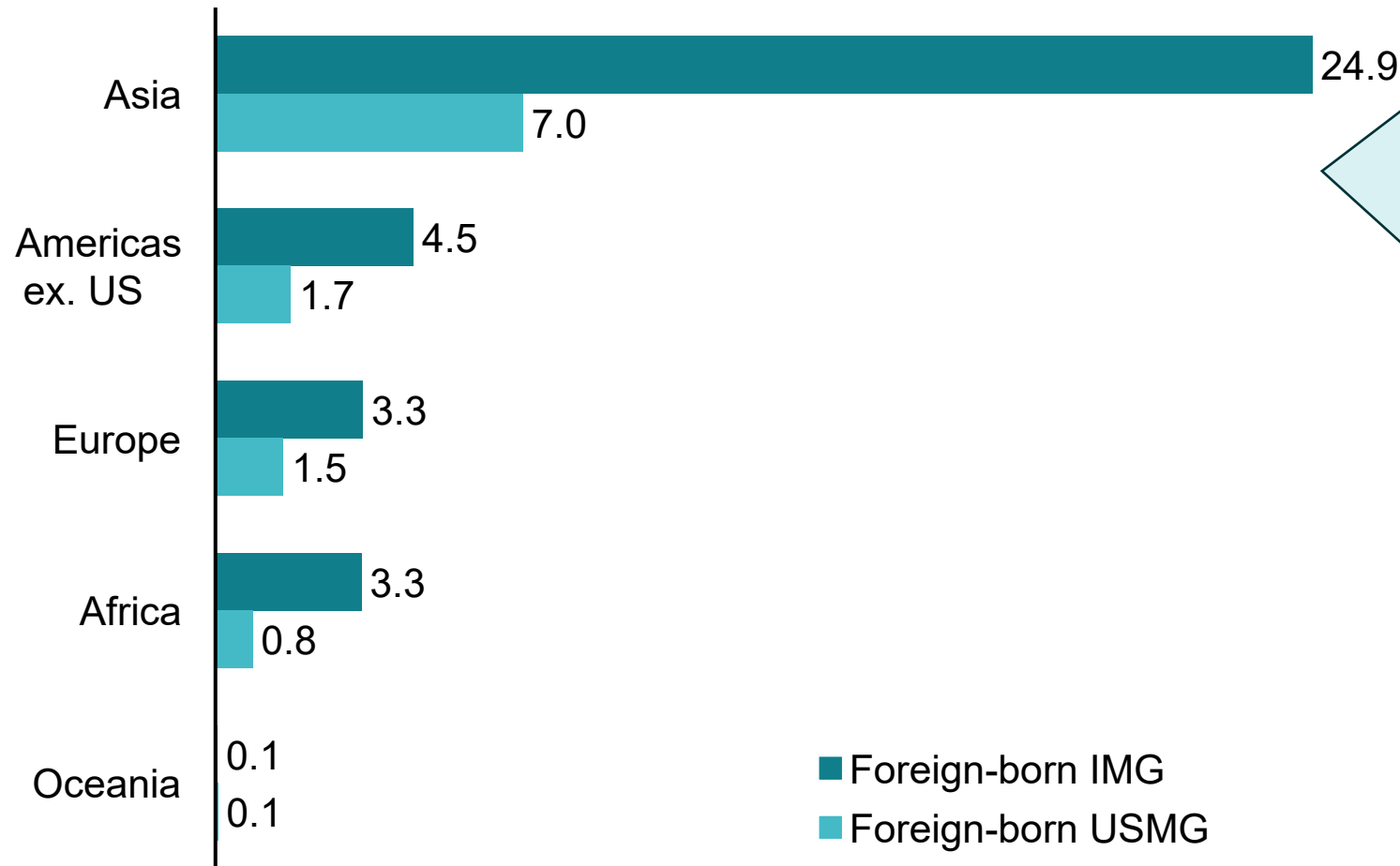


Specialties by training and birth country



Note: Physicians with multiple certifications may be in multiple categories. Most sleep medicine physicians also have CRIT or PULM certification. Specialty assigned based on ABIM certification or enrollment in a fellowship during 2025. GIM denotes general internist, i.e., no subspecialty.

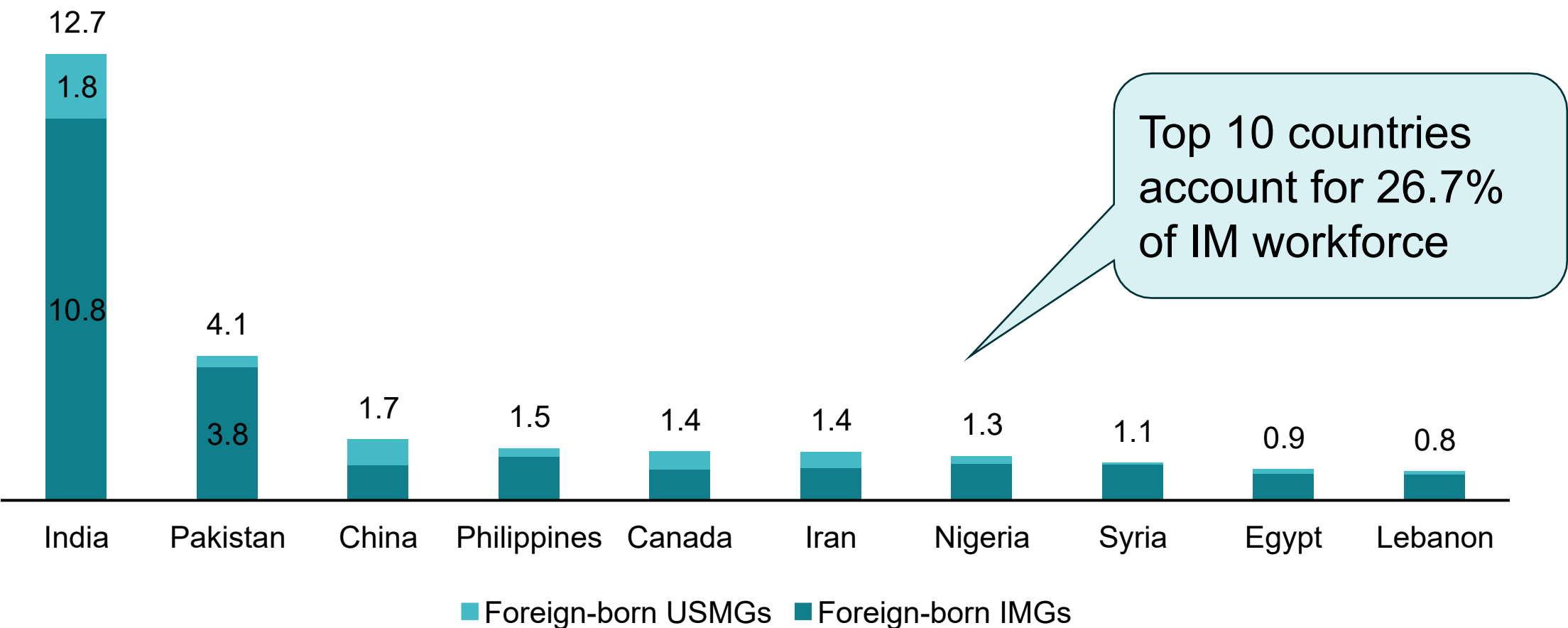
Foreign-born physicians by region of birth, % of all IM physicians



- Most foreign-born physicians from Asia, largest populations India and Pakistan (next slides)
- Totals:
 - Foreign-born IMG: 36.1%
 - Foreign-born USMG: 11.1%
 - Restricted immigration countries: 5.9% (4.7% IMG; 1.2% USMG)
 - Spanish-speaking countries: 3.6% (2.9% IMG; 0.7% USMG)

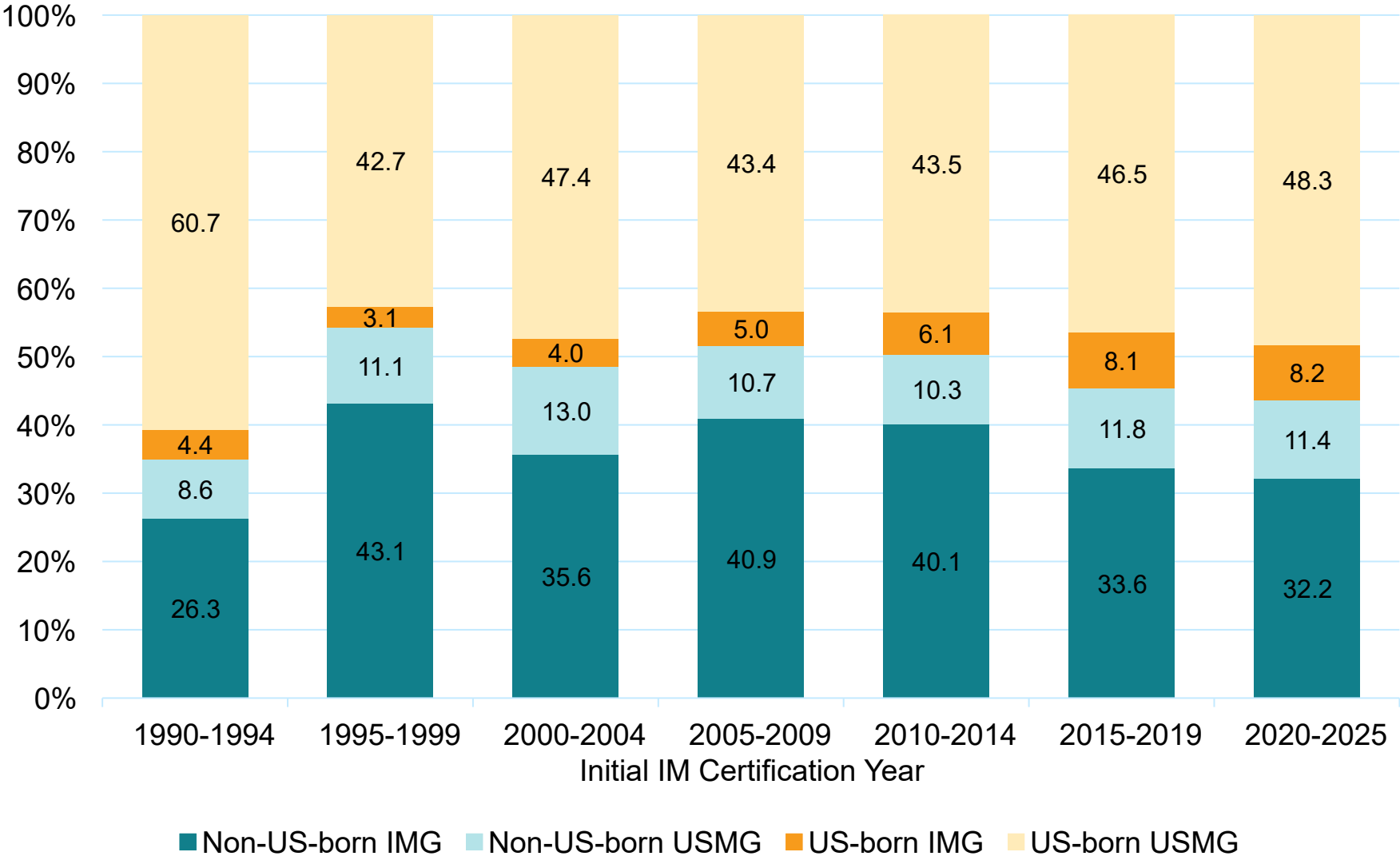
Notes: figure does not show US-born physicians (52.8% of IM physicians).

Foreign-born physicians by birth country, top 10 countries, % of all IM physicians



Note: top 10 countries by number of foreign-born IMGs; figure does not show US-born physicians (52.8% of IM physicians) or physicians born in other countries (20.5% of physicians); Iran, Nigeria, and Syria have immigration bans.

Time trends in training and birth country

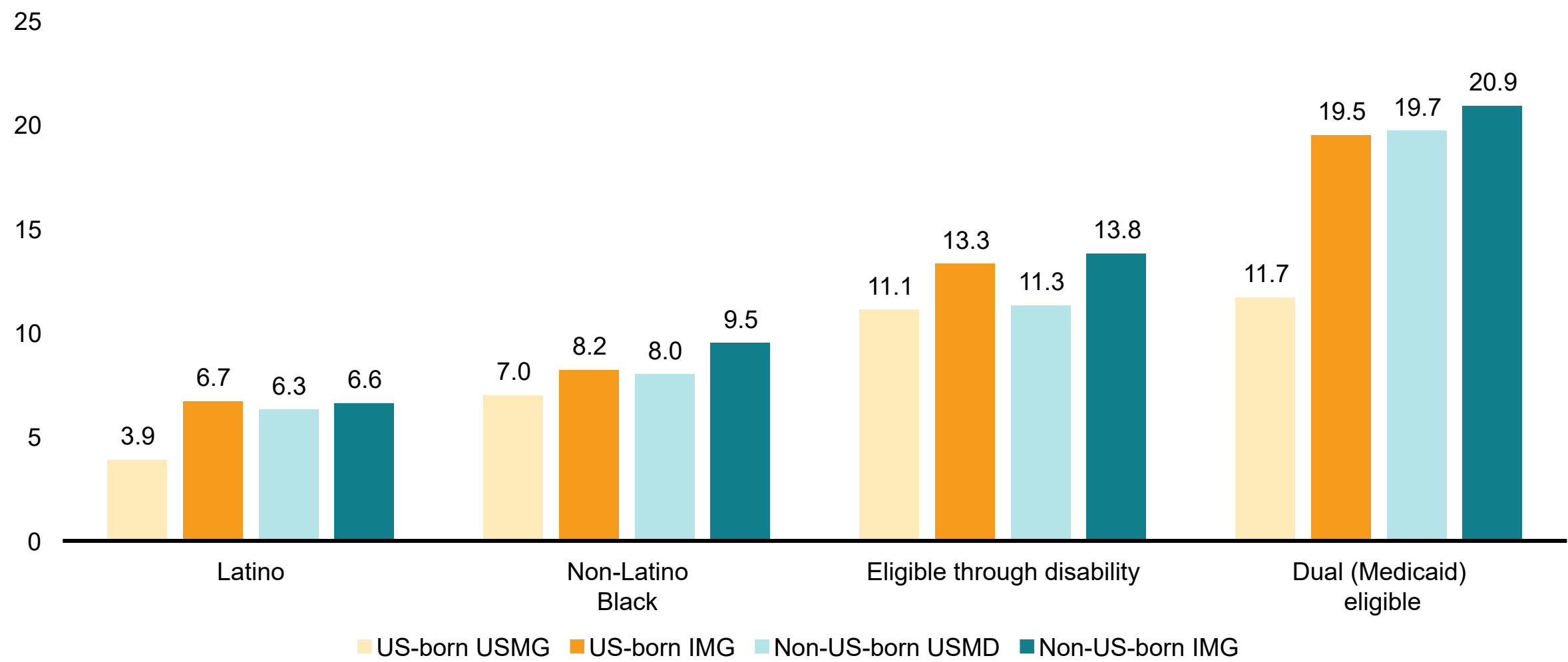


- Non-US-born IMGs grew substantially in 1990s with immigration reform
- US-born IMGs have steadily grown since 1995

Patient characteristics analysis

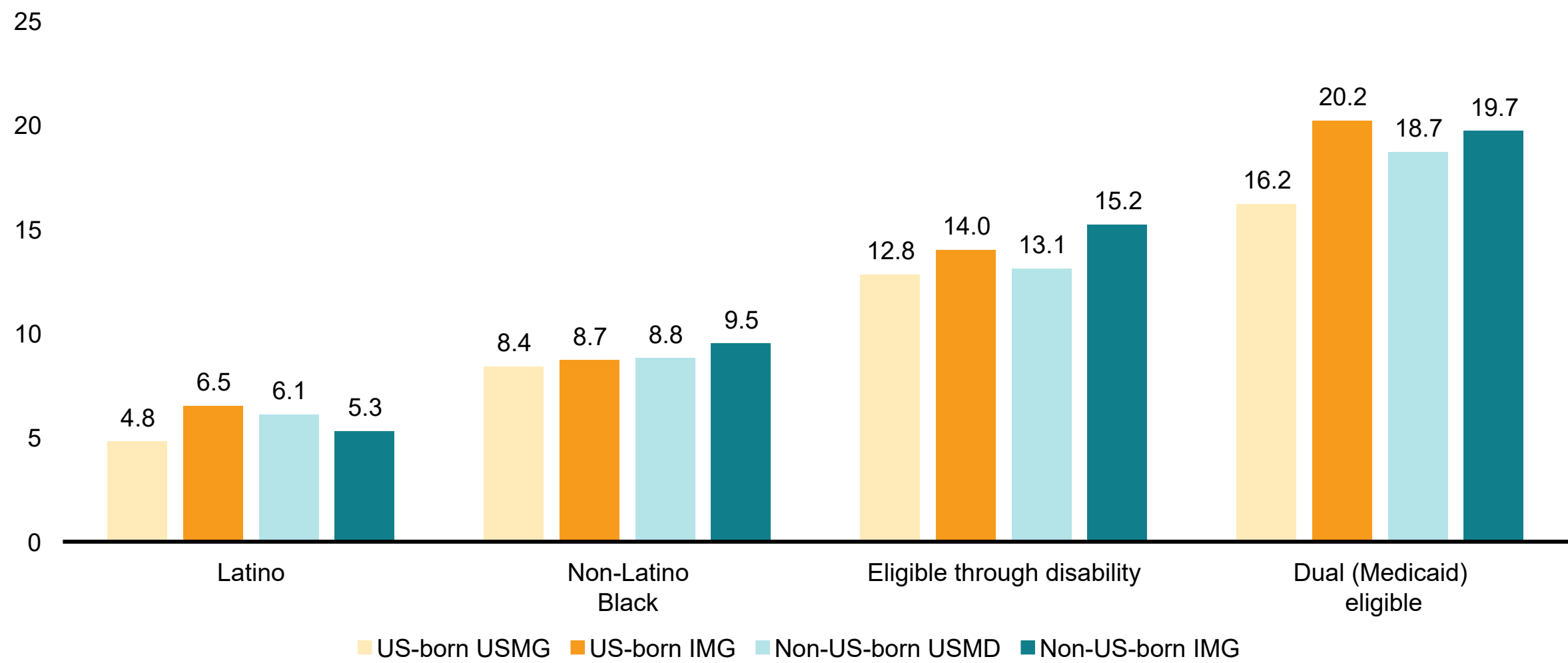
- Uses 100% Traditional Medicare claims for 2024 for patients age 65+
- All physicians initially certified in IM from 1990-2023, except those in residency/fellowship during 2024
- Count of claims that billed for E&M, procedure, or treatment
- Patient demographics and location from Medicare claims

Patient demographics by physician type, % of physician's patients



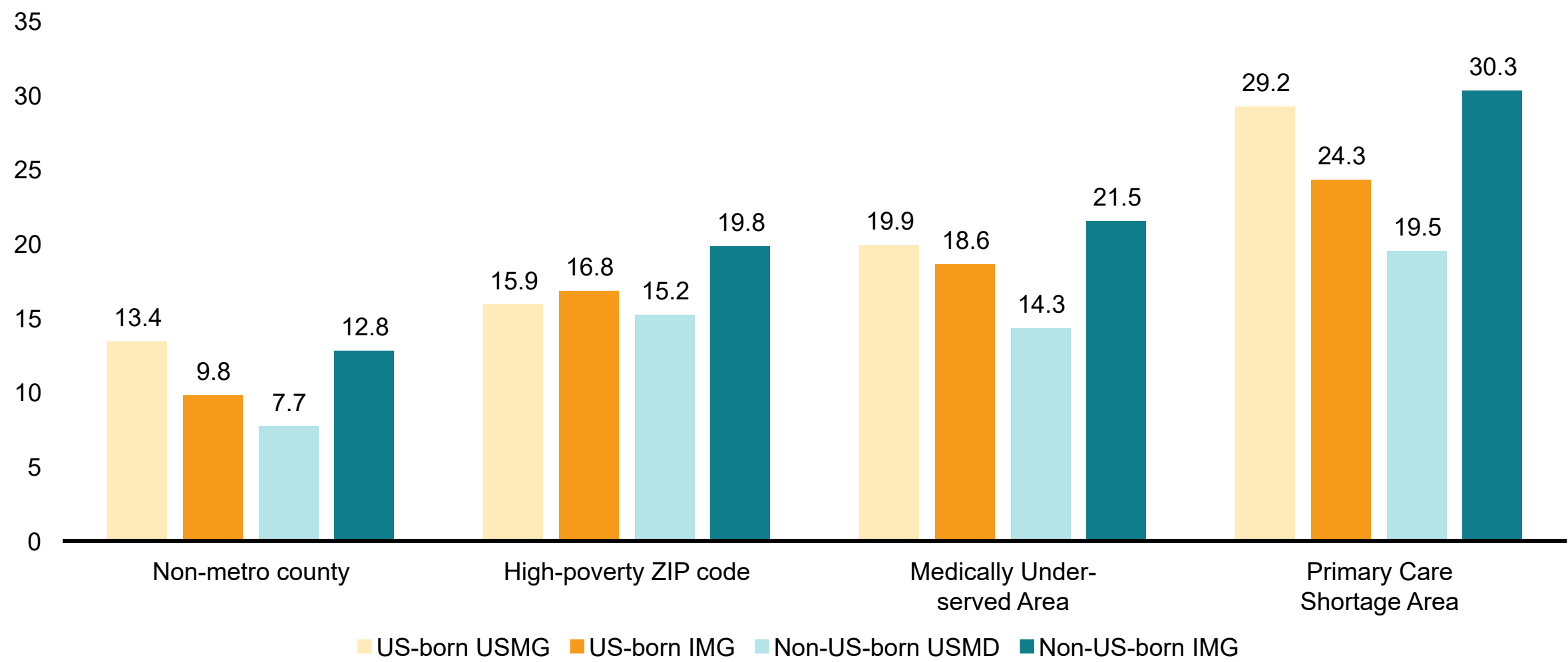
Note: Eligible for disability denotes original eligibility at time of enrollment

Patient demographics by physician type, 1-3 years after training, % of physician's patients



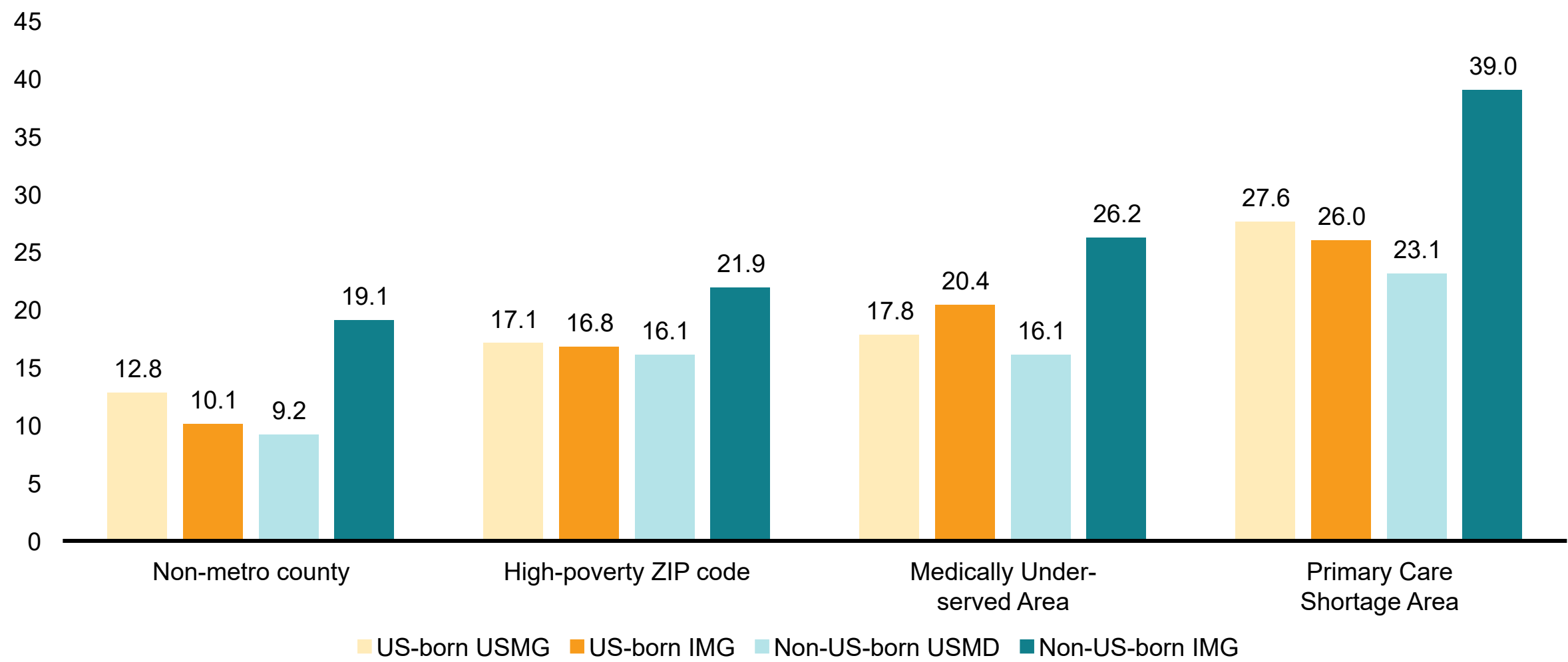
Note: Eligible for disability denotes original eligibility at time of enrollment

Patient location by physician type, % of physician's patients



Note: Eligible for disability denotes original eligibility at time of enrollment

Patient location by physician type, 1-3 years after training, % of physician's patients



Note: Eligible for disability denotes original eligibility at time of enrollment

Key findings

- Non-US-born physicians make up 47% of the workforce, non-US-born IMGs account for 36%
- Some specialties especially reliant on non-US-born physicians: non-US-born IMGs make up 53% of geriatricians, 52% of nephrologists and sleep medicine physicians, and 45% of endocrinologists
- US-born IMGs and non-US-born physicians more likely than US-born USMDs to treat Medicare patients from underserved populations
- Non-US-born IMGs, followed by US-born USMDs most likely to treat Medicare patients from underserved locations

Project status – published!

Research Letter

Non-US-Born Physicians in the US Internal Medicine Workforce

Giacomo Meille, PhD¹; Bradley M. Gray, PhD¹; Jonathan L. Vandergrift, MS¹; [et al](#)

» [Author Affiliations](#) | [Article Information](#)



CiteThis



Permissions



Metrics

JAMA

Published Online: August 12, 2026

doi: 10.1001/jama.2026.13268

Future work – effects on IM training programs, including survey of PDs